Washington State Health Care Authority

Antidiabetics – GLP-1 Agonists

Medical policy no. 27.17.00-3 Effective Date: 12/1/2025

Related medical policies:

Policy Number	Policy Name
61.25.20.AA	Anorexiants/Anti-Obesity: GLP-1 Receptor Agonists- Wegovy® (semaglutide)
61.25.25.AA	Anorexiants/Anti-Obesity: GLP-1 Receptor Agonists - Zepbound® (tirzepatide)

Note: *New-to-market drugs included in this class based on the Apple Health Preferred Drug List are non-preferred and subject to this prior authorization (PA) criteria. Non-preferred agents in this class require an inadequate response or documented intolerance due to severe adverse reaction or contraindication to at least TWO preferred agents. If there is only one preferred agent in the class documentation of inadequate response to ONE preferred agent is needed. If a drug within this policy receives a new indication approved by the Food and Drug Administration (FDA), medical necessity for the new indication will be determined on a case-by-case basis following FDA labeling.*

To see the list of the current Apple Health Preferred Drug List (AHPDL), please visit: https://www.hca.wa.gov/assets/billers-and-providers/apple-health-preferred-drug-list.xlsx

Medical necessity

Drug	Medical Necessity
Dulaglutide (**Trulicity)** Exenatide/ER Liraglutide **(Victoza)** Semaglutide (**Ozempic, Rybelsus)** Tirzepatide (**Mounjaro)**	**GLP-1 Agonists** may be considered medically necessary in patients who meet the criteria described in the clinical policy below. • Non-Preferred brand name products on the Apple Health Drug List with an A-rated generic, biosimilar or interchangeable biosimilar must also meet criteria in Non-Clinical Policy No 0001 (NC-001). If all criteria are not met, the clinical reviewer may determine there is a medically necessary need and approve on a case-by-case basis. The clinical reviewer may choose to use the reauthorization criteria when a patient has been previously established on therapy and is new to Apple Health.

Clinical policy:

Clinical Criteria	
Type 2 Diabetes without OSA or MASH. Dulaglutide (Trulicity) Exenatide/ER Liraglutide (Victoza)	Preferred GLP-1 Agonists may be approved when all of the following documented criteria are met: 1. Diagnosis of type 2 diabetes or an HbA1c ≥ 6.5 within the last 12 months; **AND** 2. The patient meets the appropriate age limit for the requested product:

Semaglutide (Ozempic, Rybelsus) Tirzepatide (Mounjaro)	a. Dulaglutide, exenatide, and liraglutide ≥ 10 years old; b. Semaglutide and tirzepatide ≥ 18 years old; **AND** 3. Drug will not be used in combination with another GLP-1 agonist or DPP4 inhibitor. Non-preferred GLP-1 Agonists may be approved when all of the following documented criteria are met: 1. Criteria 1-3 above is met; **AND** 2. The patient has history of failure, defined as inability to achieve an HbA1c of <7%, using BOTH of the following separately or simultaneously for a minimum of 90 continuous days unless not tolerated, contraindicated, or clinically inappropriate: a. Metformin at maximum or highest tolerated dose; **AND** b. Liraglutide at maximum or highest tolerated dose. If ALL criteria are met, the request will be authorized for **12 months.**
	Criteria (Reauthorization) GLP-1 Agonists may be reauthorized in patients who meet the following documented criteria: 1. Diagnosis of Type 2 diabetes**; AND** 2. Documentation showing HbA1c is ≤ 7 or has improved from baseline (prior to beginning on GLP-1 treatment); **AND** 3. Will not be used in combination with another GLP-1 agonist or DPP4 inhibitor. If ALL criteria are met, the request will be authorized for **12 months.**
Patients with T2DM and established cardiovascular disease or multiple cardiovascular risk factors who are at risk for major adverse cardiovascular events Semaglutide SQ (Ozempic)	Semaglutide (Ozempic) may be approved when all the following documented criteria are met: 1. Diagnosis of type 2 diabetes or an HbA1c ≥ 6.5 within the last 12 months; **AND** 2. The patient is ≥ 18 years old; **AND** 3. Drug will not be used in combination with another GLP-1 agonist or DPP4 inhibitor; **AND** 4. Patient has history of established cardiovascular disease as defined by the following: a. Prior stroke; **OR** b. Prior myocardial infarction; **OR** c. Symptomatic carotid artery disease as confirmed by one of the following: i. Moderate to severe carotid artery stenosis (occlusion > 50%); **OR** ii. Revascularization of the internal carotid artery; **OR** iii. One or more transient ischemic attacks with focal neurological dysfunction or transient monocular blindness; **OR** d. Symptomatic coronary artery disease (e.g. angina) as confirmed by one of the following: i. Coronary stenosis ≥ 50%; **OR**

	ii. Symptomatic Angina Pectoris; **OR** iii. Prior coronary revascularization; **OR** e. Symptomatic peripheral arterial disease (PAD) as confirmed by one or more of the following: i. Intermittent claudication with ankle-brachial index < 0.85 at rest; ii. Peripheral arterial revascularization procedure. iii. Amputation due to atherosclerotic disease; **AND** 5. Patients BMI is 27 - 30 kg/m^2 a. The patient has history of failure, defined as inability to achieve an HbA1c of <7%, using the maximum tolerated dose of liraglutide for a minimum of 90 continuous days unless not tolerated, contraindicated, or clinically inappropriate. b. Patients with BMI > 30 kg/m^2 do not require prior use of liraglutide. If ALL criteria are met, the request will be authorized for **12 months.**
	Criteria (Reauthorization) GLP-1 Agonists may be approved when all the following documented criteria are met: 1. Diagnosis of Type 2 diabetes; **AND** 2. Documentation showing HbA1c is ≤ 7 or has improved from baseline (prior to beginning on GLP-1 treatment); **AND** 3. Will not be used in combination with another GLP-1 agonist or DPP4 inhibitor. If ALL criteria are met, the request will be authorized for **12 months.**
Chronic kidney disease, to reduce the risk of sustained estimated GFR decline, end-stage kidney disease and cardiovascular death in patients with T2DM Semaglutide SQ (Ozempic)	Subcutaneous semaglutide (Ozempic) may be approved when all of the following documented criteria are met: 1. Diagnosis of Type 2 or an HbA1c ≥ 6.5 within the last 12 months; **AND** 2. Patient is 18 years or older**; AND** 3. Will not be used in combination with another GLP-1 agonist or DPP4 inhibitor; **AND** 4. Diagnosis of chronic kidney disease and one of the following is met within the last 12 months: a. Estimated glomerular filtration rate (eGFR) of 50 to 75 mL/minute with a urinary albumin-to-creatinine ratio of >300mg/g; **OR** b. eGFR of 25 to <50 mL/minute with a urinary albumin-to-creatinine ratio of >100mg/g**; AND** 5. Documentation that the patient is taking an angiotensin converting enzyme (ACE) inhibitor or an angiotensin II receptor blocker (ARB) at a maximum tolerated dose, unless contraindicated or not tolerated.

	If ALL criteria are met, the request will be authorized for **12 months.** **Criteria (Reauthorization)** Subcutaneous semaglutide (Ozempic) may be reauthorized in patients who meet the following documented criteria: 1. Diagnosis of Type 2 diabetes; **AND** 2. Documentation showing HbA1c is ≤ 7 or is has improved from baseline (prior to beginning on GLP-1 treatment); **AND** 3. Will not be used in combination with another GLP-1 agonist or DPP4 inhibitor. If ALL criteria are met, the request will be authorized for **12 months.**
T2DM and adults with moderate to severe obstructive Sleep Apnea (OSA) and obesity Tirzepatide (Mounjaro)	Tirzepatide (Mounjaro) may be approved when all of the following documented criteria are met: 1. Diagnosis of Type 2 diabetes or an HbA1c ≥ 6.5 within the last 12 months; **AND** 2. Patient is 18 years or older; **AND** 3. Patient has a body mass index (BMI) > 30 kg/m2; **AND** 4. Polysomnogram showing that the patient has moderate to severe obstructive sleep apnea as defined as an overall AHI, AHI-3%, AHI-4%, or RDI ≥ 15 obstructive respiratory events per hour (apneas, hypopneas, or RERAs), without the use of a positive airway. pressure device (PAP); **AND** 5. Patient meets one of the following criteria: a. The patient was consulted about use of PAP; **AND** b. Declined use of PAP; **OR** c. Unable to tolerate PAP therapy; **OR** d. Actively using PAP and will continue to use PAP; **AND** 6. Tirzepatide will not be used in combination with another GLP-1 agonist or DPP4 inhibitor. If ALL criteria are met, the request will be authorized for **12 months.** **Criteria (Reauthorization)** Tirzepatide (Mounjaro) may be reauthorized in patients who meet the following documented criteria: 1. Diagnosis of Type 2 diabetes; **AND** 2. Documentation showing HbA1c is ≤ 7 or is has improved from baseline (prior to beginning on GLP-1 treatment); **AND** 3. Will not be used in combination with another GLP-1 agonist or DPP4 inhibitor. If ALL criteria are met, the request will be authorized for **12 months.**
	Semaglutide SQ (Ozempic) may be approved when all of the following documented criteria are met: 1. Diagnosis of Type 2 Diabetes or an HbA1c ≥ 6.5 within the last 12 months; **AND** 2. Patient is 18 years or older; **AND**

T2DM and adults with moderate to severe (F2 or F3) noncirrhotic metabolic dysfunction associated steatohepatitis (MASH), formerly known as non-alcoholic steatohepatitis (NASH) Semaglutide SQ (Ozempic)	3. Will not be used in combination with another GLP-1 agonist or DPP4 inhibitor; **AND** 4. Patients has a diagnosis of moderate to severe MASH (Fibrosis state F2 or F3) confirmed by the following: a. biopsy; **OR** b. Fib-4 score 1.3 to 3.48; **OR** c. Fibroscan score 8.5 – 13.5 kPA; **OR** d. APRI 1.0 – 1.9; **OR** e. Fibrotest 0.49 – 0.74; **OR** f. Elastography (ARFI/PSWE) 1.38 – 2.33 m/s; **AND** 5. One or more cardiovascular risk factors listed in appendix A. If ALL criteria are met, the request will be authorized for **12 months.**
	Criteria (Reauthorization) Semaglutide SQ (Ozempic) may be reauthorized in patients who meet the following documented criteria: 1. Diagnosis of Type 2 diabetes; **AND** 2. Documentation showing HbA1c is ≤ 7 or is has improved from baseline (prior to beginning on GLP-1 treatment); **AND** 3. Will not be used in combination with another GLP-1 agonist or DPP4 inhibitor. If ALL criteria are met, the request will be authorized for **12 months.**

Appendix A:

Cardiovascular Risk Factors include (but not limited to):	
• Chronic kidney disease • Dyslipidemia • Hypertension	• Left ventricular hypertrophy • Obesity BMI ≥ 30 kg/m^2

Dosage and quantity limits

Drug	Approved Dose	Dosage Form and Quantity Limit
Dulaglutide (Trulicity)	0.75 mg SQ once weekly; after 4 weeks may increase to 1.5 mg SQ once weekly Max dose= 4.5 mg weekly	• 0.75 mg/0.5 mL pen injector: 2 mL per 28 days • 1.5 mg/0.5 mL pen injector: 2 mL per 28 days • 3 mg/0.5 mL pen injector: 2 mL per 28 days • 4.5 mg/0.5 mL pen injector: 2 mL per 28 days

Exenatide Extended Release (Bydureon BCise)	2 mg SQ once every 7 days	• 2 mg/0.85 auto injector: 3.4 mL per 28 days
Exenatide (Byetta)	5 mcg SQ twice daily then may increase to 10 mcg twice daily	• 5 mcg/0.02 mL pen injector: 1.2 mL per 28 days • 10 mcg/0.04 mL pen injector: 2.4 mL per 28 days
Liraglutide (Victoza)	0.6 mg SQ daily for 1 week. Maintenance: 1.2 mg once daily Max dose= 1.8 mg per day	• 18 mg/3 mL pen injector (3 mL package): 3 mL per 28 days • 18 mg/3 mL pen injector (6 mL package): 6 mL per 28 days • 18 mg/3 mL pen injector (9 mL package): 9 mL/ per 28 days
Tirzepatide (Mounjaro)	2.5 mg SQ once weekly for 4 weeks, then increase to 5 mg SQ once weekly. Max dose= 15 mg SQ once weekly	• 2.5 mg/0.5 mL pen injector: 2 mL per 28 days • 5 mg/0.5 mL pen injector: 2 mL per 28 days • 7.5 mg/0.5 mL pen injector: 2 mL per 28 days • 10 mg/0.5 mL pen injector: 2 mL per 28 days • 12.5 mg/0.5 mL pen injector: 2 mL per 28 days • 15 mg/0.5 mL pen injector: 2 mL per 28 days
Semaglutide (Ozempic)	0.25 mg SQ once weekly for 4 weeks, then increase to 0.5 mg once weekly Max dose= 2 mg once weekly	• 2 mg/3 mL pen injector: 3 mL per 28 days • 4 mg/3 mL pen injector: 3 mL per 28 days • 8 mg/3 mL pen injector: 3 mL per 28 days

Background:

Glucagon-like peptide 1 (GLP-1) agonists, also called incretin mimetics, are used for the treatment of type 2 diabetes. GLP-1 causes the pancreas to produce more insulin after eating and helps keep blood glucose levels within the normal range. GLP-1 agonists mimic the action of GLP-1 made by the body and can affect glucose control through several mechanisms including enhancement of glucose-dependent insulin secretin, slowed gastric emptying, and reduction of postprandial glucagon and food intake.

References

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History

Approved Date	Effective Date	Version	Action and Summary of Changes
8/18/2025	12/1/2025	27.17.00-3	• Formatting changes • Updated criteria # 5 in Patients with T2DM and established cardiovascular disease or multiple cardiovascular risk factors who are at risk for major adverse cardiovascular events • Updated Appendix A
8/18/2021	11/1/2025	27.17.00-3	• Removed t/f metformin and liraglutide if the request is for tirzepatide if patient has OSA. • Removed t/f liraglutide if the request is for Ozempic and patient has either established cardiovascular disease or chronic kidney disease. • Added DPP4 to duplicate therapy criteria. • Further defined established cardiovascular disease. • Added MACE and MASH indications for Ozempic • Added OSA indication for Mounjaro • Removed Byetta, Bydureon Bcise, Xultophy, Soliqua and Adlyxin from policy.
8/18/2021	7/1/2025	27.17.00-2	• Clarified non-preferred GLP-1 agonists need to step through liraglutide • Added criteria that requested product will not be used in combination with another GLP-1 agonist • Remove requirement to step through an SGLT2 inhibitor • Added GLP-1 combination products • Added criteria for chronic kidney disease in patients with type 2 diabetes • Updated Dosage and Quantity limits table • Updated references

Date	Action and Summary of Changes
12/20/2024	Version 1 Update: Update reauthorization criteria for Type 2 diabetes and established cardiovascular disease/risks.
02/07/2024	Version 1 Update: • Updated age indication for dulaglutide • Updated quantity limits for Byetta, Victoza, and Adlyxin
11/27/2023	Removed 2b from clinical criteria section.
09/29/2023	Version 1 Update:

	• Updated quantity limit for Ozempic • Updated medical necessity language
10/12/2022	Version 1 Update: Added Mounjaro and updated dose and quantity limits.
09/02/2021	Added established cardiovascular disease or risk of cardiovascular disease in patients with Type 2 diabetes indication
08/18/2021	Approved by DUR Board
06/14/2021	New policy created